

Medical Admission Home Care GRG

GRG: GRG-MED-HC (HC)

MCG HealthHome Care
28th Edition

This MCG guideline has not yet been updated to reflect new guidance and requirements found in the 2024 Medicare Advantage and Part D Final Rule (CMS-4201-F). For Medicare Advantage beneficiaries, before referring to MCG's guidelines users should first follow all guidance related to this topic located in CMS National Coverage Determinations (NCDs), Local Coverage Determinations (LCDs), and other statutes and regulations. Content used in MCG guidelines may be used to supplement but does not replace, modify, or supersede existing Medicare regulations or applicable NCDs or LCDs.

Note: An appropriate Optimal Recovery Guideline (ORG) should be identified and used whenever possible. This General Recovery Guideline (GRG) is intended to aid only in situations in which no ORG appears applicable.

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Clinical Indications for Admission to Home Healthcare

- Home healthcare is/was needed for appropriate care of patient because of **ALL** of the following(1)(2)(3):
 - The patient has had a face-to-face encounter with a physician or allowed non-physician practitioner (NPP) no more than 90 days prior to (or within 30 days after) the start of care related to the home health service needs.(A)(4)
 - The patient is Homebound or homebound waiver applies (eg, accountable care organization).(6)(7)
 - The patient's needs can be safely met with Intermittent home care.
 - The patient needs education on condition and self-management care with teach-back.(8)
 - Skilled care at home is required for **1 or more** of the following(9):
 - Burden of care assessment of caregiver, with potential referral to respite care(10)(11)
 - Cardiovascular status assessment(12)
 - Cast or immobilizer care(13)
 - Chest tube management(14)
 - Clinician assessment after emergency department or observation care without hospital admission
 - Coordination of care with case management program (ie, condition management, care transition)(15)(16)
 - Coordination of care with patient-centered medical home program(17)(18)
 - Coordination of care with outpatient cancer treatment center(19)
 - Coordination of care with outpatient dialysis center(20)
 - Coordination of care with outpatient infusion center(21)
 - Coordination of care with outpatient wound care center
 - Dialysis-related care, including laboratory testing(22)
 - Electrolyte management
 - Gastrointestinal status assessment
 - Genitourinary status assessment
 - Heart monitor management(23)
 - Home safety assessment(24)(25)
 - Indwelling pleural catheter management(26)
 - Infusion (IV) therapy management (eg, insertion site use and care, infusion administration, monitoring reaction)(27)
 - Jaundice level assessment(28)
 - Medication management, adherence instruction, and side effects assessment(29)
 - Mental status examination(30)

- Neurologic status assessment
- Neurovascular status assessment
- Nutrition and hydration management(31)
- Oxygen or respiratory equipment management (14)
- Pain management(32)
- Psychosocial assessment, management, and referrals(33)
- Rehabilitation therapy or equipment coordination[B][C](35)
- Respiratory status assessment
- Significant chronic condition exacerbation with need for clinical intervention and monitoring
- Skin integrity assessment(36)
- Telehealth services establishment[D](38)(39)
- Urinary catheter management(40)
- Wound or dressing management(41)

Evaluation and Treatment

General Treatment Course

Stage	Clinical Status	Interventions
1	• Clinical indications for admission met • Home care comprehensive assessment and initial home evaluation complete[E] QM(34)(42) ◦ The comprehensive assessment should be completed within 5 days after the start-of-care date. ◦ If skilled nursing is ordered, nursing should complete start of care and confirm eligibility. ◦ A qualified rehabilitation therapist (ie, PT, OT, SLP) may open and case manage home care if the orders are for therapy only (if any nursing care is ordered, the case is not considered therapy only). However, Medicare benefits do not cover cases if only occupational therapy (OT) is ordered. • Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment  SR complete for evaluation of current level of functioning (eg,  Activities of Daily Living (ADL) Scoring Tool Calculator) • Psychosocial Assessment  SR QM(43) • Social determinants of health screening complete. See Social Determinants of Health Screening Tool  SR for more information. QM • Medication reconciliation complete. See Medication Reconciliation Tool  SR for more information. QM • Home safety assessment complete. See Home Safety Assessment  SR for more information. • Hospital readmission risk factor evaluation QM (44) • Physical therapy (PT) initial home evaluation as ordered • Occupational therapy (OT) initial home evaluation as ordered • Speech language pathology (SLP) initial home evaluation as ordered • Skilled healthcare needs identified	• Establish and implement interdisciplinary care plan. QM (16)(45) • Initiate transition of care planning. QM  Provide treatments and monitoring QM (42): ◦ Adherence improvement strategies ◦ Asthma management ◦ Bladder management ◦ Bowel management ◦ Cardiac care ◦ Cast or immobilizer management ◦ Chest tube management ◦ Chronic condition management(46) ◦ Compression stocking management ◦ Dialysis management ◦ Emergency plan management ◦ Enteral nutrition management ◦ Fall risk management QM (47) ◦ Fluid and electrolyte management ◦ Glucose management ◦ Heart monitor management ◦ Indwelling pleural catheter management ◦ Infusion (IV) therapy management (eg, fluids, antibiotics, parenteral nutrition)(48) ◦ Injection medication management (ie, intramuscular, subcutaneous)

- Laboratory testing or monitoring, if ordered
- Medication management **QM**
- Nutrition management
- Oxygen management **QM**
- Pain management[F] **QM**
- Pressure injury risk management **QM**
- Psychosocial issue and risk management **QM**
- Respiratory care
- Thromboprophylaxis management
- Urinary catheter management
- Weight management
- Wound management **QM**
- Report complications to treating provider, if indicated.
- Coordinate follow-up with treating provider(s).
- Provide patient and caregiver education, reinforcement, and teach-back. **QM** (50)
 - Review patient management techniques (eg, medication, activity, monitoring).
 - Review complications and when and to whom to report.
 - See corresponding Patient Education for Clinicians for more information.

☐ Initiate referrals as indicated.

(51)

- Physical therapy (PT) referral
- Occupational therapy (OT) referral
- Speech-language pathology (SLP) referral
- Social worker referral
- Respiratory therapy referral
- Dietitian referral
- Home Health Aide (HHA) referral
- Alcohol and other drug abuse or dependence treatment, as needed **QM**
- Behavioral health support (eg, counseling)
- Community services(52)
- Educational program (eg, condition management, self-management)(15)
- End-stage disease planning
- Financial, for follow-up care, medication, and transportation

- Post subacute care services (eg, outpatient center)
- Self-help or support group
- Self-management information(53)
- Tobacco use treatment, as needed **QM** (54)
- Vocational rehabilitation(55)
- Other

- 2**
- Therapeutic response to interventions **QM**
 - Patient/caregiver able to monitor and self-manage condition, or unwilling or unable to be trained after sufficient efforts made (56)
 - Rehabilitation program established and patient participating as able with evaluation of current level of functioning (eg,  Activities of Daily Living (ADL) Scoring Tool Calculator) **QM**
 - Physical therapy (PT) progression **QM**
 - Occupational therapy (OT) progression **QM**
 - Speech-language pathology (SLP) progression **QM**

- Interdisciplinary care plan continues
 - Ongoing assessment of clinical needs
- ☐ Provide treatments and monitoring **QM** :
- Adherence improvement strategies
 - Asthma management
 - Bladder management
 - Bowel management
 - Cardiac care
 - Cast or immobilizer management
 - Chest tube management
 - Chronic condition management
 - Compression stocking management
 - Dialysis management
 - Emergency plan management
 - Enteral nutrition management
 - Fall risk management **QM**
 - Fluid and electrolyte management
 - Glucose management
 - Heart monitor management
 - Indwelling pleural catheter management
 - Infusion (IV) therapy management (eg, fluids, antibiotics, parenteral nutrition)(48)
 - Injection medication management (ie, intramuscular, subcutaneous)
 - Laboratory testing or monitoring, if ordered
 - Medication management **QM**
 - Nutrition management
 - Oxygen management **QM**
 - Pain management **QM**
 - Pressure injury risk management **QM**
 - Psychosocial issue and risk management **QM**

- Respiratory care
- Thromboprophylaxis management
- Urinary catheter management
- Weight management
- Wound management **QM**
- Continue patient and caregiver education, reinforcement, and teach-back. **QM**
 - Assess patient management ability and techniques.
 - Provide instructional support as needed.
 - Verify understanding of complications and when and to whom to report.
- Report complications to treating provider, if indicated.
- Continue transition of care planning. **QM**
- Verify follow-up with treating provider.
- Follow up on referral status, as indicated.

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- Clinical monitoring device discontinued or can be managed independently
- **Inserted or implanted device discontinued, or functioning normally and can be managed independently**
- **IV therapy discontinued or can be managed independently**
- Medical equipment and supplies available and safe use demonstrated(24)
- **Medical status stable for patient's condition(42)**
- **Medication regimen established and reconciliation completed QM**
- Outpatient center care independently coordinated by patient/caregiver
- **Patient/caregiver able to monitor and self-manage condition independently, or unwilling or unable to be trained after sufficient efforts made(56)**
- **Patient Safe at home or appropriate referrals made QM(57)**
- **Plan of care established with end-of-life/palliative provider, as appropriate for patient's condition QM**
- Psychosocial issues adequately addressed or appropriate referrals made **QM**
- **Rehabilitation program completed for safe transfer to lower level of care or patient no longer demonstrating significant functional gains (eg,**
 **Activities of Daily Living (ADL) Scoring Tool Calculator)(58)**
- **Significant chronic disease exacerbation resolved**
- **Transition plans understood**
- **Wound(s) healing without complications, or complications resolving**

- **Transition of care planning completed QM**
- Discharge from home care services

Recovery Milestones are indicated in **bold**.

Patient Education

- ☐ Patient education for medical conditions may include:
 - ☐ Medication
 - Special instructions based on underlying condition (eg, antibiotics, anticoagulants)
 - ☐ Nutrition
 - ☐ Nutritional food label reading information includes(59)(60):
 - Percent of daily values is usually based on a 2000-calorie (8374-kJ) diet.
 - Nutrition information is listed per serving, not per total contents of package.

- Packages may contain more than 1 serving.
 - Two servings double the amount of nutrients, including calories, fat, and sodium.
 - Size of serving varies from product to product.
- Form of food has impact on nutrition (eg, increased sugar content of orange juice vs one orange).
- Address nutritional needs specific to condition.

☐ Activity level

- Base exercise counseling on patient's functional abilities, treatments, and underlying condition.
- Educate patient on restrictions related to health condition.
- Return to driving or regular activity after medical clearance.

☐ Self-management

- Condition management(61)
 - Determine knowledge base, pertinent factors in providing education, and develop plan of care that is realistic for patient's abilities, desires, and resources.
- ☐ Condition education includes(8)(62):
 - Specific information about condition (eg, diagnosis, procedure)
 - Anatomy and functional impact
 - Signs and symptoms of complications
 - Whom and how to contact for questions or if complications occur
 - Health promotion or self-management activities
- Bladder and bowel management
 - Address urinary incontinence, if appropriate.
 - Assess need for bowel and bladder program.
 - Provide care and education for catheters, if present.
- Cardiac/circulatory system management
 - Blood pressure monitoring, as needed
 - Cardiac status assessment, as needed
- Central nervous system management
 - Delirium monitoring and management, as needed
- Gastrointestinal management
 - Gastrointestinal status assessment, as needed
 - Address diarrhea or constipation.
- Hygiene management
- Immunosuppression management
- Infusion management
- Musculoskeletal management
 - Provide care and education for immobilizers, orthoses, or prostheses, as needed.
 - Provide care and education on contracture prevention.
- Neuromuscular management
 - Monitor for autonomic dysreflexia, as appropriate.
- Pain management
- Prevention strategies
 - DVT prevention
 - Compression stocking use and care
 - Fall prevention
 - Infection prevention
- Respiratory management
 - Monitor and provide education on use of needed respiratory equipment (eg, spirometer, inhaler).
- ☐ Smoking cessation information includes(63)(64):
 - Benefits of quitting
 - Counseling and plan development; considerations include[G](66):
 - Willingness to quit and encouraging caregivers to quit
 - Education and formal cessation programs
 - Behavior modification
 - Medication or nicotine replacement, as appropriate(67)
 - Stress management
 - Exercise program
 - Identification of support group
 - Relapse prevention
- Skin integrity management
- Weight management
- Vital sign management
- Wound and dressing management

- Provide education and care, as appropriate.
 - Education for patients with open or chronic wounds
- Provide education on complications to report to provider based on treatments or underlying condition.
- ☐ Rehabilitation
 - Rehabilitation techniques and training(35)
- ☐ Treatment and follow-up care
 - ☐ Importance of(68)(69):
 - Keeping all follow-up appointments
 - Adherence to self-management plan and consequences of nonadherence
 - Seeking help for potential barriers that may prevent adherence to prescribed treatment plan
 - Keeping providers informed of changes in physical condition
 - Coordinate or facilitate communication regarding care plan with all appropriate care providers.
 - Follow-up care may include:
 - Age-appropriate cancer screenings (eg, cervix, breast, colon, prostate, and skin)
 - Cardiac or pulmonary rehabilitation program
 - Case management
 - Central dual-energy x-ray absorptiometry (DXA) every 2 years for osteoporosis monitoring, if indicated(70)
 - Family planning
 - Genetic counseling
 - Outpatient therapy (eg, radiation, infusion, chemotherapy)
 - Pain management
 - Participation in rehabilitation program
 - Patient-centered medical home (PCMH)(17)
 - Routine ophthalmologic, dermatologic, and dental examinations
 - Social services
 - Specialty rehabilitation center (eg, spinal cord injury, traumatic brain injury)
 - Vocational training
- ☐ Other learning needs
 - Emergency planning, as indicated
 - Recovery expectations
 - Psychosocial management
- ☐ Resources
 - Internet sources include:
 - ☐ Assistance programs to help finance prescription drug costs such as www.needymeds.org, <https://medicineassistancetool.org>, www.medicare.gov/drug-coverage-part-d, or individual pharmaceutical company websites
 - ☐ MedlinePlus, a service of the U.S. National Library of Medicine and National Institutes of Health: www.medlineplus.gov
 - ☐ Smokefree. National Institutes of Health: www.smokefree.gov
 - Related guidelines include:
 - Antibiotic Management  SR
 - Bladder Management  SR
 - Blood Sugar Management  SR
 - Caregiver Support  SR
 - Constipation Management  SR
 - Dysphagia Management  SR
 - Enteral Nutrition Management  SR
 - General Nutrition  SR
 - Hygiene Management  SR
 - Infection Prevention Strategies  SR
 - Intravenous (IV) Infusion Management  SR
 - Lifestyle Change Management  SR
 - Medication Management  SR
 - Oxygen Management  SR
 - Parenteral Nutrition Management  SR
 - Pregnancy Care  SR

- Pressure Injury Prevention and Management [SR](#)
- Quality of Life Management [SR](#)
- Safety, Preventive, and Routine Care [SR](#)
- Seizure Management [SR](#)
- Transition Planning [SR](#)
- Urinary Catheter Management [SR](#)

Discharge Planning

- Transition of care planning may include(42)(57):
 - ▢ Assessment and early establishment of anticipated discharge destination and plans for care, including:
 - Treatment plan development involving multiple providers
 - Premorbid functioning(71)
 - Patient and caregiver preferences and abilities
 - Psychosocial status. See Psychosocial Assessment [SR](#) for further information. **QM**
 - Social determinants of health screening. See Social Determinants of Health Screening Tool [SR](#) for further information. **QM**(72)
 - Acceptance for care for patient at next level of care, as appropriate
 - Transition of care plan complete, which may include:
 - Patient and caregiver education complete. See Teach Back Tool [SR](#) for further information.(8)
 - ▢ Medication reconciliation completion includes **QM** (73)(74):
 - Compare patient's discharge list of medications (prescribed and over-the-counter) against provider's admission or transfer orders.
 - Assess each medication for correlation to disease state or medical condition.
 - Report medication discrepancies to prescribing provider, attending physician, and primary care provider, and ensure accurate medication order is identified.
 - Provide reconciled medication list to all treating providers.
 - Confirm that patient or caregiver can acquire medication.
 - Educate patient and caregiver.
 - Provide complete medication list to patient and caregiver.
 - Importance of presenting personal medication list to all providers at each care transition, including all provider appointments
 - Reason, dosage, and timing of medication (eg, use "teach-back" techniques)(29)
 - Encourage communication between patient, caregiver, and pharmacy for obtaining prescriptions, setting up home medication delivery, and reviewing for drug-drug interactions.
 - See Medication Reconciliation Tool [SR](#) for more information.
 - Appointments planned or scheduled
 - Assistance and support referrals completed
 - Medical equipment and supplies coordinated (ie, delivered or delivery confirmed) as indicated
 - Transition plan communicated to all members of patient's care team **QM**
- Home architectural modifications in progress or completed, as appropriate(75)

Quality Measures

- HEDIS Measures include(76):
 - Medication reconciliation is completed.
 - Transition of care needs are assessed and addressed.
 - Psychosocial status (eg, depression screening) is assessed and addressed.
 - Fall risk is assessed and addressed.
 - Tobacco use is assessed and addressed.
 - Immunization status is assessed and addressed.
 - Readmission risk is assessed and addressed.
 - Alcohol and substance misuse is assessed and addressed.
 - Social needs screening and intervention is completed.
- Home Health Compare Quality Measures include(77):
 - ADL status is assessed for improvement.
 - Readmission risk is assessed and addressed.
 - Pressure injury risk is assessed and addressed.
 - Oral medication management is assessed for improvement.
 - Timely initiation of care is assessed and addressed.

- o Medication reconciliation and education are completed.
 - o Immunization status is assessed and addressed.
 - o Dyspnea is improved.
 - o Discharge to community is completed.
- Home Health Quality Reporting Program quality measures include(78):
 - o ADL status is assessed for improvement.
 - o Immunization status is assessed and addressed.
 - o Medication reconciliation and education are completed.
 - o Readmission risk is assessed and addressed.
 - o Fall risk is routinely assessed.
 - o Pain status is assessed and pain is addressed.
 - o Oral medication management is assessed for improvement.
 - o Pressure injury risk is assessed and addressed.
 - o Timely initiation of care is assessed and addressed.
 - o Transfer of health information to provider and patient is completed.
 - o Dyspnea is improved.
- The Joint Commission Home Care National Patient Safety Goals include(79):
 - o Medication reconciliation is completed. Education is provided on medication administration including the importance of bringing up-to-date medication lists to all provider visits.
 - o Fall risk is assessed and addressed.
 - o Home oxygen therapy safety risk is assessed.

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Footnotes

[A] **National guidelines:** The Centers for Medicare and Medicaid Services (CMS) indicates that a face-to-face encounter must occur no more than 90 days prior to the home health start-of-care date or within 30 days of the start of the home healthcare as related to the primary reason the patient requires home health services. This encounter may be performed utilizing telehealth service in an approved originating site. CMS is currently providing waivers around telehealth usage through December 31, 2024. Supporting documentation requirements include the allowed provider type, the date of encounter, the need for skilled services, the patient's homebound status, and that the encounter was related to the primary reason the patient requires home health services.(4)(5) [A in Context Link 1]

[B] **National guidelines:** The Centers for Medicare and Medicaid Services indicates that a Physical therapy (PT) referral, Occupational therapy (OT) referral, or Speech-language pathology (SLP) referral may occur once the patient is admitted to home care services by skilled nursing. If no skilled nursing services are on the home health plan of care, a PT or SLP may establish eligibility. However, an OT may establish eligibility under other programs (eg, Medicaid).(34) [B in Context Link 1]

[C] **National guidelines:** According to the Centers for Medicare and Medicaid Services (CMS), a maintenance therapy program helps to maintain the patient's current condition or to prevent or slow further deterioration. CMS specifies that skilled therapy services are necessary for the performance of a safe and effective maintenance program only when (a) the particular patient's special medical complications require the skills of a qualified therapist to perform a therapy service that would otherwise be considered nonskilled, or (b) the needed therapy procedures are of such complexity that the skills of a qualified therapist are required to perform the procedure.(4) [C in Context Link 1]

[D] **National guidelines:** The Centers for Medicare and Medicaid Services indicates that the home health agency may provide telehealth services to Medicare beneficiaries as long as the virtual visits are part of the plan of care and do not substitute for in-person visits.(37) [D in Context Link 1]

[E] **National guidelines:** The Centers for Medicare and Medicaid Services uses a data collection system to establish best practices, understand costs, and identify cost-effective systems and procedures. The current data collection system for eligible patients is the Outcome and Assessment Information Set (OASIS). OASIS is an assessment tool used to identify patient problems, strengths, and preferences. The OASIS used to create an individualized care plan can be found by going to www.cms.gov and searching OASIS Data Sets.(34) [E in Context Link 1]

[F] Naloxone rescue training and education about overdoses should be offered to the patient and family/caregiver, as indicated.(49) [F in Context Link 1]

[G] **Specialty society:** The American College of Cardiology (ACC) supports combining pharmacotherapy with behavioral/psychosocial interventions to help smokers sustain abstinence.(65) [G in Context Link 1]

Definitions

Custodial care

- Custodial care is personal unskilled care to support the patient's care of activities of daily living (ADL), such as bathing, dressing, and eating.(1)

References

1. Mauk KL. Post-acute care. In: Larsen PD, editor. Lubkin's Chronic Illness: Impact and Intervention. 11th ed. Jones & Bartlett Learning; 2022:477-520.

Dietitian referral

- Dietitian referral for high-risk condition may be considered for **1 or more** of the following(1)(2)(3):
 - Nutrition assessment
 - Detailed medical nutrition therapy (MNT) planning(4)
 - Adaptation of special diet to lifestyle or cultural habits
 - Diet order changes of:

- Therapeutic diet (eg, low-salt, low-cholesterol, diabetic)
- Parenteral or enteral nutrition
- Nutrient calculation (eg, carbohydrates, sodium)
- Failure to respond to nursing dietary interventions
- Pediatric growth or development issue

References

1. Malnutrition: overnutrition and undernutrition. In: Escott-Stump S, editor. Nutrition and Diagnosis-Related Care. 9th ed. Wolters Kluwer; 2022:790-830.
2. Harding MM. Nutrition problems. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems. 12th ed. St. Louis, MO: Mosby; 2023:1009-1029.
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4. Centers for Medicare and Medicaid Services. "Allowable administrative costs." 42 CFR Pt. 409.46 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

Home Health Aide (HHA) referral

- Home Health Aide (HHA) referral may be considered when **ALL** of the following exist(1)(2):
 - HHA supports skilled treatment plan (ie, facilitates treatment, prevents deterioration).
 - Nonskilled service needed for hands-on personal care (eg, bathing or dressing assistance), or other service to maintain patient's health (eg, repetitive practice exercises) or facilitate treatment (eg, simple dressing changes not requiring services of licensed nurse)
 - Significant functional limitations and:
 - Patient cannot perform service themselves.
 - There is no able and willing caregiver available to support patient care needs, or there is an able and willing caregiver available, but patient will not allow services to be performed by that caregiver.
 - Absence of functional plateau

References

1. Centers for Medicare and Medicaid Services. "Dependent services requirements." 42 CFR Pt. 409.45 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.
2. Centers for Medicare and Medicaid Services. "Condition of participation: Home health aide services." 42 CFR Pt. 484.80 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

Homebound

- Patients are homebound if they have functional impairments that make leaving home a considerable taxing effort or are advised by their provider not to leave home. Homebound status may not be required if there are circumstances important for home monitoring of the patient's condition (eg, patient may need clinical evaluation of home regimen of chronic disease). Pediatric patients may have different considerations for homebound status, including infection prevention, dyspnea during activity, or home confinement for medical issues.[A](1)(2)(3)

References

1. Home Health Services. Chapter 7 Rev 11447 In: Medicare Benefit Policy Manual [Internet-Only Manual] [Internet] Centers for Medicare and Medicaid Services. 2022 Jun Accessed at: <https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/Internet-Only-Manuals-IOMs-Items/CMS012673>. [accessed 2023 Sep 21]
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3. Wright EM, Sharps P, Flagg J, Busch D. Health planning for maternal-infant and child health settings. In: Savage CL, editor. Public / Community Health and Nursing Practice: Caring for Populations. 2nd ed. Philadelphia, PA: F.A. Davis; 2020:420-446.

Footnotes

- A. **National guidelines:** The Centers for Medicare and Medicaid Services indicates that a considerable taxing effort generally includes the use of supportive devices such as crutches, canes, wheelchairs, and walkers; the use of special transportation; or the assistance of another person to leave home. A patient whose absences are infrequent, for short duration (eg, religious services, graduations), or necessary for healthcare (eg, dialysis, outpatient chemotherapy, medical day care) may still be considered homebound.(1)

Intermittent home care

- Intermittent home care is defined as part-time services that are provided in the home.[A](1)(2)(3)

References

1. Home Health Services. Chapter 7 Rev 11447 In: Medicare Benefit Policy Manual [Internet-Only Manual] [Internet] Centers for Medicare and Medicaid Services. 2022 Jun Accessed at: <https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/Internet-Only-Manuals-IOMs-Items/CMS012673>. [accessed 2023 Sep 21]
2. Deutsch A. Health policy. In: Larsen PD, editor. Lubkin's Chronic Illness: Impact and Intervention. 11th ed. Jones & Bartlett Learning; 2022:569-584.
3. Community-based services. In: Chies S, editor. Pratt's Long-Term Care. 5th ed. Jones & Bartlett Learning; 2022:183-218.

Footnotes

- A. **National guidelines:** The Centers for Medicare and Medicaid Services defines intermittent care as services that must be less than 8 hours each day and 28 or fewer hours each week (or, for special cases, up to 35 hours each week).(1)

Occupational therapy (OT) progression

- OT progression includes **ALL** of the following(1)(2):
 - Ongoing evaluation of treatment plan with short-term and long-term goals
 - Ongoing evaluation of rehabilitation potential toward achieving goals
 - Patient demonstrates resolution of barriers to transition of care and **1 or more** of the following:
 - Measured improvements in performance of short-term goals in reasonable and predictable time frame based on treatment plan(4)(5)
 - Minimal progress due to unexpected clinical event, but progress expected to resume toward goals as condition improves

References

1. Bennett K. Setting rehabilitation goals. In: O'Hanlon S, Smith M, editors. Comprehensive Guide to Rehabilitation of the Older Patient. 4th ed. Elsevier; 2021:49-52.
2. Pryor J, O'Reilly K, Bonser M, Garret G, McKenchnie D. Rehabilitation for the individual and family. In: Chang E, Johnson A, editors. Living With Chronic Illness and Disability. 4th ed. Chatswood NSW 2067: Elsevier; 2022:161-182.
3. Yang W, Houtrow A, Cull DS, Annaswamy TM. Quality and outcome measures for medical rehabilitation. In: Cifu DX, et al., editors. Braddom's Physical Medicine and Rehabilitation. 6th ed. Philadelphia, PA: Elsevier; 2021:100-114.e2.
4. Kee YYK. Measuring progress with rehabilitation. In: O'Hanlon S, Smith M, editors. Comprehensive Guide to Rehabilitation of the Older Patient. 4th ed. Elsevier; 2021:111-115.
5. Barker K, Eickmeyer S. Therapeutic exercise. Medical Clinics of North America 2020;104(2):189-198. DOI: 10.1016/j.mcna.2019.10.003.

Footnotes

- A. Improvements may be measured by accuracy (percentage or number of correct trials), increased number of repetitions, decreased assistance level (maximum, moderate, or minimal assistance), decreased pain level, increased ROM/strength, increased balance scores, decreased level of cues or prompts required (in conjunction with level of assistance), or improvement in standardized functional outcome measures (eg, FIM®, WeeFIM®, Tinetti, Berg Balance Scale).(3)(4) Generally, it is expected that measured improvements should be demonstrable in targeted areas over a 1-week to 2-week period of skilled therapy. The speed of recovery is variable and may be linked to intensity of treatment, patient condition, age, comorbidities, and other factors.

Occupational therapy (OT) referral

- Occupational therapy (OT) referral may be considered when **ALL** of the following exist(1):
 - Skills of therapist required to assess functional needs and limitations, rehabilitation potential, and to develop or implement therapy plan of care(2)
 - Patient able to make measurable improvement in condition within reasonable time frame(3)
 - Need for evaluation, training, or education related to functional decline for **1 or more** of the following(4)(5):
 - Decreased independence or safety in performance of ADL or IADL. See Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment for more information.
 - Mental function changes
 - Neuromusculoskeletal and movement-related functions
 - Sensory function and pain
 - Cardiovascular, integumentary, and respiratory system function

References

1. American Occupational Therapy Association, Inc. Occupational Therapy Practice Framework: Domain & Process. 4th ed. AOTA Press, 2020.

- Centers for Medicare and Medicaid Services. "Examples of skilled nursing and rehabilitation services." 42 CFR Pt. 409.33 Washington, DC 2023 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.
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- Home care: a unique specialty. In: Marrelli TM, St. Pierre M, editors. Handbook of Home Health Standards. 6th ed. St. Louis, MO: Elsevier Mosby; 2018:1-100.
- Barker KDD, Johnson MM. The psychiatric history and physical examination. In: Cifu DX, et al., editors. Braddom's Physical Medicine and Rehabilitation. 6th ed. Philadelphia, PA: Elsevier; 2021:1-41.e2.

Oxygen or respiratory equipment management

- Specialty society:** Medical guidelines recommend that patients getting home oxygen therapy be evaluated by a nurse or licensed clinician with experience in home oxygen use within 4 weeks of home therapy initiation.(1)(2)(3)

References

- Hardinge M, et al. British Thoracic Society guidelines for home oxygen use in adults. Thorax 2015;70 Suppl 1:i1-43. DOI: 10.1136/thoraxjnl-2015-206865. (Reaffirmed 2023 Jan)
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Patient/caregiver able to monitor and self-manage condition, or unwilling or unable to be trained after sufficient efforts made

- National Guideline:** The Centers for Medicare and Medicaid Services indicates that continued teaching and training is not reasonable if the patient and/or caregiver cannot or will not be trained in an appropriate time frame. In these situations, considerations for an alternative level of care and referrals to community resources are appropriate to meet patient needs.(1)

References

- Centers for Medicare and Medicaid Services. "Beneficiary qualifications for coverage of services." 42 CFR Pt. 409.42 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

Physical therapy (PT) progression

- PT progression includes **ALL** of the following(1)(2)(3):
 - Ongoing evaluation of treatment plan with short-term and long-term goals
 - Ongoing evaluation of rehabilitation potential toward achieving goals
 - Patient demonstrates resolving of barriers to transition of care and **1 or more** of the following:
 - Measured improvements in performance of short-term goals in reasonable and predictable time frame based on treatment plan(4)(5)
 - Minimal progress due to unexpected clinical event, but progress expected to resume toward goals as condition improves

References

- Malone DJ. Introduction to physical therapist management of the acute care patient. In: Malone DJ, Bishop KL, editors. Acute Care Physical Therapy. 2nd ed. Slack, Inc.; 2020:1-50.
- Bartels MN, Prince DZ. Acute medical conditions: cardiopulmonary disease, medical frailty, and renal failure. In: Cifu DX, et al., editors. Braddom's Physical Medicine and Rehabilitation. 6th ed. Philadelphia, PA: Elsevier; 2021:511-534.e5.
- Centers for Medicare and Medicaid Services. "Skilled services requirements." 42 CFR Pt. 409.44 Washington, DC 2023 Oct [accessed 2023 Oct 04] Accessed at: <https://www.ecfr.gov/>.
- Yang W, Houtrow A, Cull DS, Annaswamy TM. Quality and outcome measures for medical rehabilitation. In: Cifu DX, et al., editors. Braddom's Physical Medicine and Rehabilitation. 6th ed. Philadelphia, PA: Elsevier; 2021:100-114.e2.
- Barker K, Eickmeyer S. Therapeutic exercise. Medical Clinics of North America 2020;104(2):189-198. DOI: 10.1016/j.mcna.2019.10.003.

Footnotes

- Improvements may be measured by accuracy (percentage or number of correct trials), increased number of repetitions, decreased assistance level (maximum, moderate, or minimal assistance), decreased pain level, increased ROM/strength, increased balance scores, decreased level of cues or prompts required (in conjunction with level of assistance), or improvement in standardized functional outcome measures (eg, FIM®, WeeFIM®, Tinetti, Berg Balance Scale).(1)(4) Generally, it is expected that measured

improvements should be demonstrable in targeted areas over a 1-week to 2-week period of skilled therapy. The speed of recovery is variable and may be linked to intensity of treatment, patient condition, age, comorbidities, and other factors.

Physical therapy (PT) referral

- Physical therapy (PT) referral may be considered when **ALL** of the following exist(1):
 - Skills of therapist required to assess functional needs and limitations, rehabilitation potential, and to develop or implement therapy plan of care(2)
 - Patient able to make measurable improvement in condition within reasonable and predictable time frame(3)
 - Need for evaluation, training, or education related to functional decline for **1 or more** of the following(4):
 - Aerobic capacity and endurance
 - Ambulation or mobility
 - Balance or coordination
 - Contracture management (education, splint, positioning)
 - Decreased independence or safety in performance of ADL or IADL. See Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment for more information.
 - Developmental progression (ie, pediatric developmental delay)(5)
 - Durable medical equipment (DME) or orthotic device management
 - Environmental and home barriers
 - Ergonomics and body mechanics
 - Functional mobility (eg, bed and transfer training)
 - Level of care planning (eg, home exercise program vs outpatient therapy)
 - Motor function (motor control and motor learning)
 - Muscle performance (range of motion or strength)
 - Pain or edema management
 - Self-care management
 - Wound management

References

1. Malone DJ. Introduction to physical therapist management of the acute care patient. In: Malone DJ, Bishop KL, editors. Acute Care Physical Therapy. 2nd ed. Slack, Inc.; 2020:1-50.
2. Centers for Medicare and Medicaid Services. "Examples of skilled nursing and rehabilitation services." 42 CFR Pt. 409.33 Washington, DC 2023 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.
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Rehabilitation program completed for safe transfer to lower level of care or patient no longer demonstrating significant functional gains

- **National guideline:** Rehabilitation services are not considered skilled for the supervision of taught exercises, repetitious exercise administration (unless there is a loss of function), or are provided for assistance only.(1)

References

1. Centers for Medicare and Medicaid Services. "Examples of skilled nursing and rehabilitation services." 42 CFR Pt. 409.33 Washington, DC 2023 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

Respiratory therapy referral

- Respiratory therapy referral may be considered when **1 or more** of the following are needed^{[A](1)(2)}:
 - Respiratory treatment (eg, nebulization, suctioning) administration and education
 - Oxygen or mechanical ventilation monitoring, adjustment, and education
 - Maintenance therapy to design, establish, implement, and evaluate program that contributes to maintaining patient's present level of functioning

References

1. Centers for Medicare and Medicaid Services. "Allowable administrative costs." 42 CFR Pt. 409.46 Washington, DC 2022 Oct [accessed 2023 Oct 30] Accessed at: <https://www.ecfr.gov/>.

2. Mondor EE. Supporting ventilation. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:539-576.

Footnotes

- A. Respiratory therapist interventions depend on state regulations and the scope of the practice.

Safe at home

- Home safety may be determined by a completed Home Safety Assessment and the patient being in no imminent harm. Harm includes high risk for deterioration or injury and may include risk of harm to self or others.(1)(2)

References

1. Laplante N. Home care safety. In: Perry AG, Potter PA, Ostendorf WR, editors. *Nursing Interventions and Clinical Skills*. 7th ed. Elsevier; 2020:842-862.
2. Ekroos RA, Traveller L. Violence and abuse. In: Rector C, Stanley MJ, editors. *Community and Public Health Nursing Promoting the Public's Health*. 10th ed. Wolters Kluwer; 2022:506-536.

Skilled care

- Skilled care is that which must be performed or supervised by professional or technical personnel, be reasonable and necessary for patient's treatment, and exceed scope of Custodial care. Skilled care may be direct (eg, in-person assessment and administration of treatments) or indirect (eg, coordinating care between providers, supervising care aides).[A](1)(2)(3)(4)

References

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Footnotes

- A. **National guidelines:** The Centers for Medicare and Medicaid Services defines skilled care as a service that is "so inherently complex that it can only be safely and effectively performed by, or under the supervision of, professional or technical personnel."(1)

Social worker referral

- Social worker (SW) referral may be considered when **1 or more** of the following exist[A](3)(4):
 - Social or emotional problems that impede medical treatment or recovery(5)
 - Abuse or neglect suspected
 - Advance care planning (eg, end-of-life care, advance directives)
 - Financial or legal assistance, transportation, or housing referrals needed
 - Decision-making facilitation needed within family or among caregivers

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Footnotes

- A. Social worker (SW) subspecialties include medical, mental health, and substance use. The type of SW referral should include consideration of subspecialty, diagnosis of the patient, and the purpose for referral.(1)(2)

Speech-language pathology (SLP) progression

- SLP progression includes **ALL** of the following(1)(2)(3):
 - Established treatment plan with short-term and long-term goals(4)
 - Ongoing evaluation of rehabilitation potential toward achieving goals(5)
 - Patient demonstrates resolving of barriers to transition of care and **1 or more** of the following(5):
 - Measured improvements in performance of short-term goals in reasonable and predictable time frame based on treatment plan[A](2)(6)
 - Minimal progress due to unexpected clinical event, but progress expected to resume toward goals as condition improves

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Footnotes

- A. Improvements may be measured by accuracy (percentage or number of correct trials), increased number of repetitions, decreased assistance level (maximum, moderate, or minimal assistance), decreased pain level, increased ROM/strength, increased balance scores, decreased level of cues or prompts required (in conjunction with level of assistance), or improvement in standardized functional outcome measures (eg, FIM®, WeeFIM®, Tinetti, Berg Balance Scale).(5)(6) Generally, it is expected that measured improvements should be demonstrable in targeted areas over a 1-week to 2-week period of skilled therapy. The speed of recovery is variable and may be linked to intensity of treatment, patient condition, age, comorbidities, and other factors.(2)(5)(6)

Speech-language pathology (SLP) referral

- Speech-language pathology (SLP) therapy referral may be considered when **ALL** of the following exist:
 - Skills of therapist required to assess functional needs and limitations, rehabilitation potential, and to develop or implement therapy plan of care(1)
 - Patient able to make measurable improvement in condition within reasonable time frame(2)
 - Patient needs evaluation, training, or education related to functional decline for **1 or more** of the following(3)(4):
 - Speech
 - Language
 - Voice
 - Swallowing or chewing
 - Hearing as it relates to communication(5)(6)
 - Cognitive-communication
 - Developmental progression (ie, pediatric developmental delay)(7)
 - Augmentative/alternative communication system development, modification, training, and use(2)

References

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Telehealth services

- Telehealth services involve a remote exchange of health data between patient and provider. The data are exchanged in real time, using equipment that may send data via the internet, audio interaction, or video conferencing. The types of services delivered could include medical or rehabilitation examination, assessment, condition management, care coordination, or education.(1)(2)

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Transition of care planning completed

- Transition of care planning completed, including **ALL** of the following(1)(2):
 - Transition plan communicated to all members of patient's healthcare team
 - Summary of care, discharge list of medications, and transition plan communicated to primary care provider
 - Medication reconciliation complete
 - Education on condition management and complications to report provided to patient or caregiver[A]
 - Follow-up appointments scheduled
 - Referrals to continue medical and rehabilitation goals (eg, outpatient) arranged, if needed
 - Services (eg, equipment, environment modifications, transportation) arranged, if needed
 - Psychosocial issues addressed, or plan for management, if needed

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Footnotes

- A. Health education materials should be given in patient's and caregiver's native language using trained language interpreters whenever possible.(3)

Transition plans understood

- Transition plans and education understood, including(1)(2):
 - Patient and caregiver show ability and understanding to manage condition and treatments.
 - Patient and caregiver know signs and symptoms of complications, whom to contact, and how and when to seek medical intervention.

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